

National Youth Strategy

Palestine Member Association

2026-2028

Version 3

A loving home for every child

Document Control

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Related Documents

- Youth Care Development Guideline
- Child Protection Policy – SOS Policy
- Formal Education Policy – SOS Policy
- The Youth Care Co-Worker – HRM Concept
- Child and Youth Safeguarding Policy
- National Reintegration Guide

Change History

Version	Date / Period	Key Change
1	1 March – 1 September 2017	New care approach
2	2021	Updated care approach
3	2026	New strategic directions

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1. Executive Summary

This National Youth Strategy sets out the strategic direction of SOS Children's Villages Palestine for working with adolescents and young people during the 2026–2028 period. It reflects the evolution of the care model in Palestine, the strategic shift toward shorter and more targeted alternative care, the strengthening of family and community-based approaches, and the continued commitment to youth empowerment, participation, safeguarding, education, employability and independent living.

The strategy applies to young people in alternative care, focusing on those in Small Group Homes, and to young people supported through direct family empowerment, community empowerment and emergency response programming. It provides a common framework for programme teams, caregivers, case managers, coordinators, project managers and other co-workers to ensure that support to youth is rights-based, child-centred, participatory, protective and oriented toward sustainable outcomes.

The document builds on earlier youth care approaches and introduces clearer alignment with the current strategic direction of SOS Children's Villages Palestine. It recognizes that young people in Palestine face compounded challenges related to conflict, insecurity, restricted mobility, economic hardship, education disruption, limited labour market opportunities and psychosocial distress. At the same time, it recognizes their resilience, leadership potential and ability to contribute meaningfully to their families and communities when they are supported with the right opportunities.

Strategic Orientation

Strategic Area	Core Intention
Care and protection	Provide safe, nurturing and rights-based care for adolescents in alternative care settings, with the best interests of the child as the primary consideration.
Transition and reintegration	Ensure that movement between care settings and reintegration into biological, extended or foster families is planned, gradual, participatory and approved by relevant authorities.
Education and employability	Support young people to complete education, access vocational and career pathways, and develop the life and professional skills needed for decent work.
Youth participation	Enable young people to influence decisions affecting their lives at individual, family, community and external representation levels.
Safeguarding and wellbeing	Strengthen awareness, prevention, reporting and response systems to protect children and youth from abuse, neglect, exploitation and harm.

2. Glossary

Parental Care:

Primary responsibility for the upbringing and development of the child lies with the child's care-giver(s). This may be the biological parent(s), a member of the extended family, an SOS mother/parent or another person who takes the parental role. This role may be shared, according to family composition and cultural appropriate practices.

Alternative Care:

Care provided to children by caregivers other than their biological parents. Such care may take the form of formal or informal care. One form of alternative care is care for kinship, sponsorship and other child-care and family-based care and care in independent accommodation facilities for supervised children

SOS Family:

SOS families are foster families for children who have lost parental care and cannot live with their families of origin, and for whom a foster family is the best option. SOS families are headed by a SOS mother, organised in a community and supported by the SOS Children's Village.

Small Group Home:

A community-based alternative care model that provides a small, safe residential environment for adolescents, normally six to eight young people. It is based on rights-based care principles and the best interests of the child, with 24-hour care and support provided by a team of trained caregivers.

Kinship Care:

The placement of a child with extended family members, older siblings, or, in some cases, family friends. This arrangement may be formal or informal.

Youth Development:

The physical, emotional, intellectual, social, cultural and spiritual growth and progress of children and young people from childhood to young adulthood.

Development Planning:

A comprehensive planning process used by SOS Children's Villages to understand the development of each young person and define short-, medium- and long-term objectives across health, education, professional preparation, social development, emotional development, life skills and values.

Education:

The holistic development of knowledge, understanding and application, including values, competencies and identity formation through formal schooling and informal learning linked to family, social life and community context.

Reintegration:

The process through which a child or young person separated from their family returns permanently to their family and community, where this is safe, appropriate and in their best interests.

Reunification:

The return of a child to the family where possible and consistent with the best interests of the child. Reunification involves more than physical return; it requires preparation, support, safety assessment and follow-up.

Youth Empowerment:

The creation and support of enabling conditions under which young people can act on their own behalf and on their own terms. This includes access to knowledge, information, skills, resources, supportive systems and a positive value base.

Youth Employability:

Education, training, guidance and preparation that support young people to access decent work and build stable, independent lives.

Youth Care Co-Worker:

A co-worker is responsible for guiding youth toward independence according to SOS Children's Villages policies and procedures. The role includes providing advice, guidance, support and positive role modelling from adolescence to adulthood.

3. Acronyms

Acronym	Meaning
CFE	Community Family Empowerment
DFE	Direct Family Empowerment
FLC	Family-Like Care
MHPSS	Mental Health and Psychosocial Support
NEET	Not in Education, Employment or Training
SGH	Small Group Home
SIL	Supervised Independent Living
SOS CV	SOS Children's Villages
YCCW	Youth Care Co-Worker

4. WHO WE ARE

SOS Children's Villages Palestine is a humanitarian, non-governmental, and non-profit organization established in Palestine in 1966. Its mission is to care for children who have lost parental care and to prevent family breakdown through various programs and interventions, including family strengthening, youth care, education, and emergency response to the needs of children and families.

The organization focuses on preventing family separation through its Family Strengthening Program, which provides social, psychological, and economic services that help families achieve financial independence and remain united so they can care for their children. When this is not possible, the organization provides alternative family care in Children's Villages, where children grow up in a loving and safe environment with a caregiver and a supportive family.

SOS Children's Villages also supports adolescents and young people through education, vocational training, empowerment and employment programs, and psychosocial support—enabling them to become self-reliant and active, positive participants in their communities. The organization encourages higher education, as well as the development of life and professional skills that help them enter the job market and participate in community initiatives.

In Palestine, the SOS Children's Village in Bethlehem was built in 1968 and today host 133 children in 24 homes. Another village was established in Rafah in 2000, but it was destroyed during the war on Gaza in May 2024. The 68 children living there were evacuated to Bethlehem, and an SOS camp was established to care for about 90 additional children inside the Gaza Strip.

Amid war and famine, the organization has provided shelter and care for unaccompanied children, emergency education, cash and in-kind assistance, healthcare, psychosocial support, and more to over 58,000 people, including 32,000 children, and is working with 1,600 children to bring them back to school in Gaza.

5. Introduction and Strategic Rationale

Previous SOS family-based models of long-term care included the main stages of development throughout the life of a child under SOS Children's Villages care: life in an SOS family or community house, youth care, reintegration, youth facility support, leaving care and independent life.

The National Youth Strategy has historically provided a solid working base for co-workers in the Family-Like Care programme by outlining the orientations of SOS Children's Villages Palestine regarding child and youth care across these stages. It supported co-workers to understand transition processes, responsibilities, monitoring arrangements and the mechanisms needed to guide young people gradually toward independence.

Child and youth care is an ongoing process that requires close follow-up. Any gap in care, protection, psychosocial support, education, preparation or transition planning can significantly affect the development and wellbeing of children and young people. For this reason, the strategy emphasizes structured mechanisms and clear roles for caregivers, case managers, project managers and other relevant staff.

With the changes in the care model adopted by SOS Children's Villages globally, and in alignment with the United Nations Guidelines for Alternative Care, which emphasize that alternative care should be appropriate, necessary and provided for the shortest suitable period, the current version of the strategy reflects the strategic direction of SOS Children's Villages Palestine for 2026–2028.

This includes the planned downsizing and refocusing of alternative care services, the strengthening of family and community-based approaches, and the expansion of youth-focused work within alternative care, direct family empowerment, community empowerment and emergency response projects. The strategy maintains the core pillars of earlier youth strategies, including empowerment, participation, safeguarding, employability, and transition to self-reliance.

Strategic Logic of the Youth Care Model

Stage / Programme Area	Main Focus	Expected Direction
Alternative care / Small Group Homes	Safe, structured, rights-based care and individualized support for adolescents.	Temporary protective care with transition planning.
Reintegration / family-based options	Prepared and monitored return to biological, extended or foster family where safe and appropriate.	Sustainable family-based care and belonging.
Supervised Independent Living	Final phase of support toward independent life for eligible young people.	Currently frozen for 2026–2028 due to lack of planned transfers.

DFE and CFE youth	Life skills, employability, participation and empowerment of young people in family and community programmes.	Preparation for education, employment and positive community participation.
Emergency response youth work	Protection, psychosocial support, education continuity and youth engagement in crisis contexts.	Resilience, safety and continuity of support.

6. Youth Reality in Palestine

Young people aged 14–29 in Palestine represent a large and vital segment of society, bridging adolescence and adulthood. They face interconnected challenges shaped by prolonged conflict, economic hardship and restricted opportunities, all of which affect their transition into independent and productive lives.

Key Challenges

Early vulnerability and disrupted transitions: Adolescents aged 14–17 often experience unstable pathways to adulthood due to exposure to violence, movement restrictions and insecurity. This increases the risk of school dropout, child labour and early marriage, especially for girls, while safe developmental spaces remain limited.

Education under pressure: Although access to education is relatively high, quality and continuity remain major concerns. Overcrowded classrooms, double-shift systems and conflict-related disruptions affect learning outcomes. Weak links between education and labour market needs leave many graduates without relevant skills.

High unemployment and economic exclusion: Youth unemployment remains very high, particularly in Gaza. Many young people are not in education, employment or training, while others rely on informal and unstable work. This delays financial independence and undermines social stability.

Psychosocial and mental health challenges: Young people face high levels of stress, trauma and anxiety due to ongoing conflict and uncertainty. Feelings of hopelessness are common, while access to mental health and psychosocial support services remains limited, especially for adolescents.

Restricted mobility and limited opportunities: Movement restrictions limit access to education, employment and social engagement, reducing exposure to wider opportunities and constraining entrepreneurship and innovation.

Limited participation but strong potential: Despite these challenges, Palestinian youth show resilience through community engagement, volunteering and informal leadership. They are digitally connected and motivated to contribute to development, but opportunities for meaningful participation in decision-making remain limited.

7. Strategic Priorities

Priority	Strategic Direction
Holistic adolescent development (14-17)	Prevent school dropout, provide safe spaces, strengthen life skills and address risks such as child labour and early marriage, while integrating social and emotional learning.
Education-to-employment pathways (18-29)	Strengthen early career guidance, expand vocational training and apprenticeships, and promote digital and remote work opportunities.
Economic empowerment	Support youth entrepreneurship through mentoring, incubation and income-generating opportunities, while engaging the private sector.
Mental health and psychosocial support	Integrate MHPSS across youth programming, expand access to services and strengthen peer support approaches where appropriate.
Youth participation and leadership	Create platforms for youth engagement, support youth-led initiatives and build leadership and advocacy skills.
Gender and inclusion	Address barriers facing young women and ensure inclusion of marginalized youth, including rural youth, youth with disabilities and young people outside education, employment or training.
Protection and safe environments	Strengthen protection systems, ensure safe physical and digital spaces, and raise awareness of rights and protection mechanisms.

8. History of SOS Children's Villages Palestine and Youth Care Evolution

Bethlehem Children's Village

The SOS Children's Village in Bethlehem was established in 1966 and was the first SOS Children's Village in the Middle East. To date, more than 500 formers

beneficiaries in the West Bank have graduated from SOS programmes, with 95% successfully integrated into the labour market.

Rafah Children's Village

The SOS Children's Village in Rafah was established in the Gaza Strip in 2001. A total of 82 youth have graduated from the programme as former SOS beneficiaries. Currently, there is no SOS Children's Village operating in Rafah, as the village was destroyed as a result of the war on Gaza.

In 2016, the reintegration programme for children and youth was initiated to support their gradual return and adaptation to their biological families and surrounding communities.

Evolution of Youth Facilities in Palestine

The establishment of youth facilities outside the SOS Children's Village in Bethlehem initially faced resistance from some co-workers and the local community, mainly due to stigma toward children and youth born out of wedlock. Despite this, the first youth facility was opened inside the village in 1982, accommodating 18 young people aged 14-17.

In 1988, SOS Children's Villages Palestine adopted the approach of integrating youth facilities within the community by opening rented houses for young people with the support of youth leaders, including the first girls' youth facility in Bethlehem.

In 1996, the girls' facility was relocated to the SOS Children's Village school, where a dedicated floor housed eight girls. Between 2006 and 2008, youth facilities for boys and girls were established in Rafah, while the growing number of beneficiaries increased demand for additional facilities in Bethlehem. In 2012, two houses were rented outside the Rafah village to enhance the social integration of boys.

SOS Children's Villages Palestine also supported young people in the semi-independence phase, aged 18 and above, who were enrolled in universities or vocational training. These young people received support until completing their studies and securing stable employment, while living independently and remaining under close follow-up by youth care co-workers.

In 2017, the reintegration process for children and youth was launched in line with the strategic plan. By 2022, all youth houses had been closed, and young people were reintegrated with their families.

Following the closure of youth facilities, and based on a study conducted by SOS Children's Villages Palestine to assess children's needs and the most suitable care model, it was identified that some children in alternative care required a different model from the Family-Like Care model. In response, SOS Children's Villages Palestine established Small Group Homes in 2025 and developed related guidance and standards for working with children and adolescents in this model.

9. Small Group Homes (SGHs)

Definition and Description

A Small Group Home is a residential care arrangement in the community that provides structured, rights-based care for children and adolescents. It is designed to be small in scale, typically accommodating no more than six to eight children or young people, and is organized around the individual rights, needs and best interests of each child.

The environment is intentionally designed to resemble, as far as possible, a family setting or a small supportive group context. The primary objective is to provide temporary protective placement while actively supporting the reintegration of the child or adolescent into their biological family. Where reintegration is not possible, the approach focuses on securing stable long-term care within an alternative family-based environment or another appropriate care arrangement.

The model is designed for children and young people above the age of 12. Care is provided on a 24-hour basis by trained caregivers who are employed and compensated for their roles. Caregivers operate within a team-based structure and work in shifts to ensure continuous, consistent care and supervision. Throughout the provision of care, the rights, dignity and developmental needs of each child or adolescent are systematically upheld.

10. Transition and Reintegration Framework

10.1 Transition from SOS Family Homes to Small Group Homes

The transition of adolescents from SOS family homes to small group homes is a structured and child-centered process designed to ensure continuity of care while promoting gradual independence. This process is grounded in comprehensive assessment, participatory planning, and phased implementation to support adolescents in navigating this critical developmental stage.

The transition process begins with a formal nomination and assessment led by the caregiving team. Adolescents are identified based on their developmental readiness, behavioural profile, and individual needs. A holistic review of each adolescent's case is conducted, including care plans, psychosocial assessments, educational performance, and social relationships. Based on this review, individualized transition objectives are defined to guide the process and ensure alignment with the adolescent's best interests.

Psychosocial preparation is a core pillar of the transition framework. Adolescents receive structured individual and group-based support aimed at strengthening emotional readiness, addressing concerns, and fostering a sense of security. Particular emphasis is placed on developing life skills, self-reliance, and adaptive capacities required for semi-independent living. Orientation processes are implemented to familiarize adolescents with the small group home environment, including its routines, expectations, and relational dynamics.

A phased exposure approach is adopted to facilitate gradual adjustment. Adolescents are provided with opportunities to visit and engage with the new environment prior to relocation, allowing them to build familiarity with caregivers, peers, and daily life arrangements. Throughout the process, adolescents are recognized as active participants, with their perspectives, preferences, and concerns systematically incorporated into decision-making.

The framework ensures continuity in education and access to services through coordinated planning with relevant institutions. Where appropriate, biological families are engaged to maintain supportive relationships and ensure transparency in the transition process. Administrative and logistical arrangements are managed in a coordinated manner, including the transfer of records, preparation of personal belongings, and documentation of readiness.

Post-transition support is an integral component of the framework. A defined follow-up period is implemented to monitor adjustment, provide ongoing psychosocial support, and review individual care plans. This ensures early identification of challenges and reinforces stability within the new living arrangement.

10.2 Reintegration from Small Group Homes to Biological Families

Reintegration into biological families is approached as a gradual, carefully managed process aimed at achieving sustainable family-based care while safeguarding the child's wellbeing. This process is guided by the principles of the best interests of the child, family strengthening, and long-term stability.

The process of integration with the biological family or any other entity is carried out in full partnership with the relevant ministry and after obtaining their final approval.

The process is initiated through a comprehensive, multidisciplinary assessment that evaluates the child's readiness for reintegration alongside the capacity of the biological family to provide a safe, stable, and nurturing environment. Particular attention is given to addressing any prior risks, including neglect, violence, or instability, to ensure that reintegration does not compromise the child's safety or development.

An individualized reintegration plan is developed as a central guiding tool. This plan outlines clear objectives, timelines, roles, and responsibilities, and defines the scope of support required for both the child and the family before, during, and after reintegration. The implementation of this plan follows a phased approach that gradually rebuilds the relationship between the child and the family.

Structured contact is progressively introduced, beginning with remote communication and advancing to supervised visits, short-term stays, and eventually longer periods within the family environment. This gradual progression is essential to mitigate emotional stress, build trust, and support a stable transition.

Therapeutic and capacity-building interventions are integrated throughout the process. Children receive tailored psychosocial support to process their experiences and prepare emotionally for reintegration, while families are supported through guidance and counseling to strengthen parenting skills, communication, and their ability to respond to the child's needs effectively.

Children and adolescents are actively engaged in the reintegration process, with their views and preferences respected in accordance with their age and maturity. Efforts are made to maintain stability in education and social integration by coordinating with schools and community structures, while minimizing disruptive changes.

Post-reintegration support is systematically provided through ongoing monitoring, home visits, and access to psychosocial services. Mechanisms are established to ensure continued support and responsiveness to emerging needs. In parallel, contingency planning is incorporated to address potential risks, including the possibility of temporary alternative care arrangements if reintegration proves unsustainable.

10.3 Structure, Roles and Responsibilities

Role	Key Responsibilities
Youth Caregivers	Provide daily supervision, build trusting relationships with adolescents, implement activities, support daily life routines, and contribute to the safe and supportive environment of the Small Group Home.
Case Manager	Review assessments, prepare and monitor individual plans, coordinate with the multidisciplinary team, engage directly with the child/adolescent and family, and ensure follow-up and documentation.
Youth Care Coordinator	Monitors the progress of plans with all youth staff, ensuring their implementation and alignment with budgets; follows up on administrative matters with other units and ensures coordination with other coordinators; builds the capacity of youth staff and ensures the implementation of all SOS policies.
Psychologist / MHPSS Professional	Conduct psychological assessments, provide therapy or psychosocial support sessions, contribute to case planning, and support emotional wellbeing and trauma-informed care.
SOS Mother / Primary Care Figure	Where relevant, maintain a stable and supportive relationship with the young person, share knowledge and experience with the care team, and support transition from family care to the Small Group Home.
Project Manager / Programme Director	Provide strategic leadership and management at location level, monitor programme quality and performance, ensure optimal use of resources, and

	support coordination across relevant teams.
Relevant Ministry and Authorities	Provide formal approval and coordination for reintegration or transfer processes where required by national procedures and legal frameworks.

Follow-up standard: In each Small Group Home, a team of three to four youth caregivers follows up with six to eight young people, providing care, educational support, health insurance follow-up and daily supervision. The specific staffing model should be reviewed against approved HR and programme standards.

10.4 Approach and Intervention Model

The Small Group Home model is grounded in a therapeutic, relationship-based philosophy that prioritizes trust, safety and human connection as drivers of behavioural change. Unlike traditional models that rely on control and punishment, this approach focuses on stable, supportive relationships between adolescents and caregivers to foster belonging, rebuild trust and address the impact of past adverse experiences such as neglect, rejection or violence.

The environment is designed to be safe, participatory and development-oriented, enabling adolescents to express themselves while being guided toward positive behavioural and social outcomes. The work modality is structured yet flexible, combining daily routines with targeted psychosocial and developmental interventions.

Daily and weekly sessions contribute to consistency and emotional stability while reinforcing personal responsibility and social values. These sessions provide space for adolescents to reflect on their emotional state, review progress and set achievable short-term goals, contributing to self-awareness and self-regulation.

Psychosocial support is provided through integrated individual and group interventions. Individual sessions are tailored to each adolescent's needs and may address anxiety, trauma, low self-esteem and family-related challenges. Group sessions focus on peer interaction, shared experiences, empathy, communication and mutual support.

Complementary Activities

- Sports activities that promote physical wellbeing, discipline and teamwork.
- Artistic and creative activities that enhance self-expression and emotional processing.
- Recreational and group-based activities that strengthen social bonds and engagement.
- Education and vocational development support, including re-engagement in formal education or alternative learning pathways.
- Vocational training and career exploration aligned with the interests and capacities of each adolescent.

Life Skills Development

- Communication, problem-solving and decision-making.
- Emotional regulation and stress management.
- Healthy relationships and respect for boundaries.
- Daily living skills and basic financial literacy.
- Leadership, participation and social responsibility.

Case Management

- Comprehensive assessment covering psychosocial, educational, behavioural and health aspects.
- Development and periodic review of an individualized development plan with clear goals.
- Ongoing monitoring, documentation and adaptation of interventions.
- Coordination with schools, health services, social protection systems and other external stakeholders.
- Transition planning embedded throughout the care process to prepare for reintegration, alternative care or independent living pathways.

11. Supervised Independent Living

Supervised Independent Living is the final phase of SOS child and youth care. Its overall objective is to support a smooth transition to fully independent life and enable the young person to assume responsibility for managing their own life.

This phase usually follows the youth care phase and may last for a maximum of four years, after which SOS Children's Villages withdraws care and support and the young person is considered fully independent.

From the beginning of 2026 to the end of 2028, this phase will be frozen due to the lack of beneficiaries planned for transfer. During this period, SOS Children's Villages Palestine will continue to operate through a methodology of integration with biological, extended or foster families where appropriate.

Origin families are referred to the Family Strengthening Unit three to six months before the planned move, to ensure that the child or young person is prepared in coordination with the biological family and relevant programme teams.

12. Youth in DFE and CFE

Young people aged 13–22 in targeted Direct Family Empowerment and Community Empowerment programmes will be supported according to the principle of preparing them for youth employment and positive participation in society.

Support will focus on equipping young people with the skills, knowledge, confidence and opportunities needed to enter the labour market smoothly, continue education or training, and contribute constructively to family and community life.

13. Youth Empowerment and Employability

Finding employment and decent work is key for young people to become self-reliant members of their communities. Young people around the world face significant challenges when transitioning into independence. They are particularly vulnerable to slow economies and economic downturns, and they are often insufficiently prepared to position themselves in labour markets.

Lack of work can put young people at risk of poverty, social exclusion and exploitation. Communities and societies also suffer when young people are unable to become contributing members of society.

Youth unemployment rates are disproportionately high in many contexts. When young people do work, employment is often poor quality and does not provide adequate stability or income security. Key barriers include weak education outcomes, limited career counselling, lack of soft skills and mismatch between skills and labour market requirements.

These challenges are particularly serious for young people without parental care or at risk of losing it, as they often cannot rely on parental guidance, networks or financial support. They may also leave care earlier than their peers and need to reach financial independence more quickly.

Empowering youth and strengthening employability is a strategic goal for SOS Children's Villages. The organization's global Strategy 2030 includes a focus on empowering young people by strengthening their care and employability. Priority areas include skills and employability training, IT and language skills, coaching, mentoring, internships and partnerships.

14. SOS Children's Villages Palestine Response

SOS Children's Villages Palestine works with children and young people of different ages to empower them and prepare them for a successful and independent future. The focus is on personal development, skill enhancement, career counselling and guidance, and access to employment experience so that young people can develop and use their full potential.

Young people must be included in decisions that affect their lives. SOS Children's Villages Palestine promotes the participation of children and young people in shaping their futures across its programmes.

Main Pillars of Youth Empowerment

Pillar	Description
Quality education	Support disadvantaged children and youth to access relevant quality education and acquire the skills, capacities and knowledge needed to establish themselves in the labour market.
Employment and entrepreneurship training	Provide practical training opportunities, including vocational, technical, language, digital and entrepreneurship skills that respond to labour

	market needs.
Access to work experience through partnerships	Build partnerships with local businesses, NGOs, community organizations and corporate partners to provide internships, job shadowing and on-the-job training.
Advocacy for equal access to employment	Promote the right of young people, including those leaving care, to equal access to decent work and supportive employment systems.
Youth-led initiatives	Guide and support young people to design and implement community-based initiatives, including initiatives focused on climate action, gender equality, disability inclusion and social enterprise.
Economic empowerment	Support selected young people through small income-generating projects, mentoring and follow-up, particularly in light of the difficult economic situation and high youth unemployment in Palestine.

15. Youth Participation

Youth participation is a purposeful and structured process that ensures the meaningful and safe involvement of children and young people in decisions and programmes that affect their lives, in ways that are appropriate to their age, capacities and development.

Participation is grounded in the right of children and young people to express their views freely, to be listened to and to have their perspectives taken into account across contexts, including alternative care, family settings and the community.

At the individual and family level, youth participation is reflected in young people's engagement in daily family life, their contribution to personal, educational and professional decisions, and their active participation in the development, implementation and follow-up of individual plans. This fosters critical thinking, responsibility and independence.

At the community level, participation includes engagement in community activities, volunteer work and the design and implementation of youth-led initiatives that respond to local needs in education, health, environment and social fields. Such engagement builds leadership, teamwork, initiative and social responsibility.

At the external level, young people should be empowered to represent themselves and their communities in conferences, seminars and workshops at local and international levels. This enables them to exchange experiences, gain exposure to diverse perspectives, build networks and amplify their voices.

To ensure sustainability, youth-led representative structures such as youth councils, children's parliaments and youth centres should be established and activated with direct youth involvement. These bodies provide safe and structured platforms for expression, dialogue and accountability, while strengthening leadership and organizational capacities.

16. Child and Youth Safeguarding

SOS Children's Villages Palestine is fully committed to ensuring a safe, nurturing and protective environment for all children and youth under its care. The organization upholds zero tolerance toward any form of child or youth abuse, neglect or exploitation, whether actual, suspected or attempted.

The Child and Youth Safeguarding Policy is built on four core pillars: awareness, prevention, reporting and response. These pillars ensure that safeguarding is embedded in programmes, systems and daily practices.

The organization promotes a culture of openness and accountability, where children, youth, staff, families and communities are encouraged to speak up about safeguarding concerns without fear of retaliation. The best interests and safety of the child remain the guiding principles in all safeguarding decisions.

Preventive measures focus on safe organizational environments through strict recruitment and vetting procedures, continuous staff training, clear professional boundaries and child participation in matters affecting their lives. SOS Children's Villages Palestine actively empowers children and youth, strengthens families and works with communities to reduce risks and promote positive care practices.

Clear and accessible reporting mechanisms are established at programme and national levels. All safeguarding concerns must be reported promptly and handled with strict confidentiality and a need-to-know approach. Designated safeguarding focal persons and multidisciplinary teams ensure coordinated case management and follow-up in line with national legislation and SOS international standards.

All safeguarding reports trigger an appropriate response proportionate to the seriousness of the concern. Allegations involving staff are managed with due process, including suspension when necessary, referral to competent authorities in cases of criminal suspicion and provision of psychosocial support to affected individuals.

Safeguarding incidents are systematically documented and monitored. Aggregated data and annual safeguarding reports support organizational learning, accountability and continuous improvement of safeguarding practices.

Through this policy, SOS Children's Villages Palestine reaffirms its commitment to transparency, child rights and high safeguarding standards, ensuring that the dignity, safety and wellbeing of every child and young person are protected at all times.

17. Implementation and Monitoring Considerations

Effective implementation of this strategy requires strong coordination among programme teams, caregivers, case managers, project managers, Family Strengthening staff, safeguarding focal points and relevant external stakeholders.

The strategy should be translated into annual operational plans with clear activities, responsibilities, indicators, timelines and resources. These plans should cover alternative care, reintegration, family strengthening, youth empowerment, employability, participation and safeguarding.

Monitoring should combine individual case follow-up, programme-level data, feedback from young people and caregivers, and periodic learning reviews. Findings should inform adaptation of interventions and continuous improvement of youth care practice.

Suggested Monitoring Areas

Area	Examples of What to Monitor
Care quality	Safety, stability, individualized planning, caregiver follow-up, access to services and child/youth satisfaction.
Education and employability	School continuity, vocational training, career guidance, job shadowing, internships, employment readiness and income-generating initiatives.
Participation	Youth involvement in individual plans, family decisions, programme design, youth-led initiatives and representative structures.
Reintegration and transition	Readiness assessments, ministry approvals, phased contact, family strengthening support, post-reintegration monitoring and contingency planning.
Safeguarding	Awareness activities, prevention measures, reporting channels, response timelines, case documentation and learning from incidents.

18. Final Approval Checklist

- Confirm final version number and strategy period with senior management.
- Verify all beneficiary numbers, dates and current statistics before external circulation.
- Confirm whether the term “Family-Like Care” or the updated official care terminology should be used consistently throughout the document.
- Confirm whether references to Rafah, emergency response figures and current Gaza operations should remain in this strategy or be moved to a separate contextual annex.
- Confirm roles and job titles, especially Youth Caregiver, Case Manager, Psychologist/MHPSS Professional, Project Manager and Programme Director.

- Ensure the document is formally aligned with approved SOS Children's Villages Palestine policies and global SOS standards.
- Obtain final approval from the National Director and relevant programme leadership before circulation.

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